

CORY SHEPHERD

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Supporting statement – two applications for sick leave without pay

Recoding of two periods presently recorded as leave without pay, at the request of Metro South Payroll · 11 September 2026

1. What is applied for

Two applications for **sick leave without pay**, for the two periods identified by Metro South Payroll on 7 and 8 September 2026. The dates are Payroll's own and are not varied.

Application	Period	Days (inclusive)
1	21 June 2024 – 20 September 2024	92
2	13 December 2024 – 23 February 2025	73

The periods are presently recorded as leave without pay. Nothing else is sought by these applications, and no other period is affected.

2. Payroll has already identified the correction and the step required

On 7 September 2026 at 1:28 pm Metro South Payroll wrote to the line manager:

“Could I please confirm with you whether the following periods were intended to be recorded as Sick No Pay rather than Leave Without Pay? 21.06.2024 - 20.09.2024 / 13.12.2024 - 23.02.2025. This will affect the employee's LSL date... we would require a leave form to be submitted for both periods. Could you please confirm?”

On 8 September 2026 at 2:24 pm Ms Grace Strachan, Senior Payroll Officer, wrote to me:

*“We believe that the periods 21/06/2024 - 20/09/2024 and 13/12/2024 - 23/02/2025 **should have been booked as Sick No Pay (SNP) rather than Leave Without Pay (LWOP)**... If these periods need to be changed to Sick No Pay, new leave forms will be required for both periods. Once the leave forms are received and the leave is updated accordingly, **we can recognise the first three months of each period towards your LSL date.**”*

These applications are made so that the leave forms Payroll requires are on foot. I am not asking anyone to accept a new position; the position is Payroll's own.

3. Application 1 – 21 June 2024 to 20 September 2024

The absence was medical from the outset and is documented throughout:

- My leave record shows **sick leave on 7, 8 and 9 June 2024**, including sick leave without pay on 9 June, and leave without pay from 10 June 2024.
- A **medical certificate was issued on 28 June 2024**.
- The **work capacity certificate of Dr Peter Hawes dated 1 July 2024** records the date of injury as **18 June 2024** and that I was first seen for that injury on 1 July 2024.
- That certificate certifies **no functional capacity for any type of work over the period 1 July to 6 October 2024** – a period which contains the whole of this application from 1 July onward.
- Further work capacity certificates were issued on **7 August 2024 (Dr Ki Pang)** and on **11 August and 8 September 2024 (Dr Hawes)**.

- Review Decision 69983 records that the certificate of 1 July 2024 indicated “there was no pre-existing factor or condition”, and that this “was maintained in all later work capacity certificates”.
- My application for workers’ compensation was **lodged on 1 July 2024** and rejected on 13 September 2024. That rejection is under appeal and is not a matter for this application.

4. Application 2 – 13 December 2024 to 23 February 2025

The absence was a continuation of the same medical condition:

- **On 24 October 2024** my treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah, diagnosed **Major Depressive Disorder with anxiety state** at the first consultation, and confirmed that diagnosis in writing the same day.
- **On 13 February 2025** – within this period – Dr Krishnaiah provided a report recording the continuing diagnosis, its severity and functional effect, and the treatment then in place.
- The period ends on **23 February 2025**, being the day before a return to work was confirmed for **7:00 am on 24 February 2025**.

If a work capacity certificate specific to this period is required in addition to what is already held, please let me know and I will obtain it from the treating practice. I would rather be told what is missing than have the application held.

4A. The medical evidence is already held in myHR

Medical certificates for these absences were provided at the time and are held in myHR against the leave records for these periods, including certificates submitted as photographs. A copy of the work capacity certificate of Dr Peter Hawes signed 8 September 2024 is attached to this statement as an example. Further copies can be provided on request.

If certificates were provided and are held against these periods, the question of how the periods came to be recorded as leave without pay rather than sick leave without pay is the question I raised with Payroll on 7 September 2026, and it remains open. **Nothing in these two applications depends on that question being answered**, and I do not ask that it be answered before the recoding is effected.

5. The effect, on Payroll’s own figures

Payroll’s calculation of 4 September 2026 records my commencement with Queensland Health as **25 March 2019**, and that eligibility would ordinarily arise on **25 March 2026**. Payroll has identified **189 days presently excluded**, being 24 days (18 May – 10 June 2022), 92 days (21 June – 20 September 2024) and 73 days (13 December 2024 – 23 February 2025), which moves the date to **29 September 2026**.

On Payroll’s stated approach – that on recoding “the first three months of each period” are recognised – each of these two periods falls **entirely** within its own first three months: three months from 21 June 2024 ends 21 September 2024, and the period closes on 20 September; three months from 13 December 2024 ends 13 March 2025, and the period closes on 23 February. On that arithmetic **165 of the 189 days return**, leaving only the 24 days of 2022, and the resulting date falls in **April 2026**. The calculation is of course Payroll’s to make.

6. Reservation

I have received no wages for work since 13 July 2026. Since 3 July 2026 I have been certified fit for duties with restrictions on the Employee Capability Checklist completed by Metro South Health, I have remained available, and I have not been provided with work. What I have been paid since 13 July has been drawn from my own leave accruals and not paid as wages.

Any leave applied for, taken or paid in that period is applied for and taken under protest and without prejudice to my claim for wages for the period I have not been provided work, and I reserve the right to

seek reinstatement of the leave taken. These two applications relate to 2024 and 2025 and are made solely to give effect to the recoding Payroll has identified; they are not an acceptance of any position about 2026.

Signed

A handwritten signature in black ink, appearing to be 'Cory Lea Shepherd', written over a horizontal line.

Cory Lea Shepherd
Administration Officer, Switchboard Services, Logan Hospital
Employee no. 388372

Date: 11 September 2026

Attached: work capacity certificate of Dr Peter Hawes signed 8 September 2024 (2 pages), as an example of the certificates already held in myHR against these periods. Further certificates, including those of Dr Ki Pang dated 7 August 2024 and Dr Peter Hawes dated 1 July and 11 August 2024, can be provided on request.

Part A - Patient details

Name: **Cory Sheppard** Date of Birth: **11/01/1981**
 Mobile number: **0833408027** Claim number: **8076440201** ☐ New claim ☐ Claim is report only
 Occupation (if known): **Admin worker** Patient's employer: **Qld Health**

Part B - Injury details

Date of accident: **11/08/2024** Patient's stated date of injury: **08/09/2024** Patient was first seen at this practice/hospital for this injury/condition on: **01/07/2024**

The patient is/was suffering from: ☒ All relevant related diagnoses ☐ Professional diagnosis only, with 'Thoughtful' diagnosis ☐ Professional diagnosis only, with 'Thoughtful' diagnosis
 Patient's chief mechanism of injury: **Violent act at work** Is this consistent with your clinical findings? ☒ Yes ☐ Unclear
 Injury/breaking of workplace rules by patient, violating him

Describe symptoms in detail
 Presenting symptoms in detail
 Aggravated (not previously suffered)

Part C - Treatment plan

Patient requires treatment for: **08/09/2024** to **08/10/2024** to be reviewed again on: **08/10/2024** ☐ No further review
 Treatment: **Self care** How this treatment
 I have provided medication that may impact safe work, travel or cognitive function: ☐ No ☒ Yes
 Medication: **Paracetamol** ☒ Allergies: **None** Home discipline: **Psychiatrist** Details (specify):

Part D - Capacity for work

☐ The condition will not prevent a return to work ☐ If suitable duties available, can return to some form of work ☐ No functional capacity for any type of work until
 Pre-injury duties: **Self care** Complete part B, Q4 to Part F, from

Complete the form section if you certified no functional capacity for any type of work
 If no functional capacity, state why? **Physical injury** Estimated time to return to some form of work duties: **Unknown** Estimated time to return to full duties: **Unknown**

Part E - Functional ability

Describe in writing the patient's functional ability, state limitations, list to complete. No change since last certificate
 Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do either at work or home.

Function/task (patient's usual)	Is functional ability affected by injury/condition?	What patient can do (if 'yes' is selected)
Functional ability	☒ Yes	Note any restrictions (if relevant)
Lower limb	☒ Yes	
Upper limb	☒ Yes	
Hand function	☒ Yes	
Spinal function	☒ Yes	
Cognition/psychosocial functioning	☒ Yes	
Driving a car	☒ Yes	
Operating machinery/vehicle	☒ Yes	
Manual tasks	☒ Yes	
Other	☒ Yes	

Part F - Rehabilitation at work - return to work plan

Options for emergency medical/psychosocial services: **None** (patient is not to complete)
 What workplace modifications are required to facilitate return to work? **None** (see assessment, psychosocial/psychiatrist)

Other considerations or factors that may affect recovery: **None** (see assessment, psychosocial/psychiatrist)

☐ I require a suitable duties program to be provided to me for approval
 I have discussed injury requirements and return to work with the patient and: ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (for use with hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.
 Name: **Peter Haines** Email: **phaines@nswmedicare.com.au**
 Practice/hospital: **Our Medical Ashmore** Phone: **0754478000** Date: **08/09/2024**
 Postal Address: **555 Glen Avenue, Ashmore**
 QLD: **4214**
 Signature: *[Signature]*

Further information: **www.worksafe.qld.gov.au/medicalsupport**
 All enquiries (medical/dental/nurse practitioner, patient, employer) call 131 122

© 2005 the Workforce Compensation and Rehabilitation Act 2005 (the Act). The evidence of certification is provided to assist the information on the form is provided by the medical/dental/nurse practitioner. The information provided on the form may be different to the patient's employer, insurer, or rehabilitation provider. It is not a requirement to complete it. The form is provided to assist in the assessment of work capacity. It is not a requirement to complete it. The form is provided to assist in the assessment of work capacity. It is not a requirement to complete it.



